

KRA — the healthcare channel that just opened

PREPARED 2026-07-03 SOURCE: Islam's KRA lunch debrief, verified against KRA's published record

INTERNAL

1 · WHAT HAPPENED

Islam had lunch with **KRA — Kirsten Reite Architecture**: Vancouver-based, **90% healthcare** by their own framing, 100+ healthcare environments across BC, Alberta, and Saskatchewan — including the **New Edmonton Hospital and Health Campus**, the Chinook Regional Hospital Addition, and the Haida Gwaii Hospital. They said they're open to collaborating with KOR in Alberta, they need a **healthcare portfolio** from KOR (prime-consultant submissions require past relevant experience for every consultant on the team), and — unprompted — they pointed at a lane:

"You have a nice spot for commissioning,
because others don't want to do it."

KRA, over lunch — 2026-07-03

They also urged speed on the **Fraser Health prequalification** ("staged, takes a long time — proceed asap") and offered to connect Islam with their **Vancouver BD manager** for non-health BC work.

2 · WHY THIS CHANNEL MATTERS

They sit where structural seats get decided

On owner-procured and prime-consultant health work, **the architect assembles the consulting team** — exactly the seat-selection channel in KOR's playbook. A healthcare-specialist architect with Alberta projects and a Vancouver head office is a **two-province door**: Alberta health collaboration now, BC non-health collaboration through their Vancouver BD manager. And their role on the New

Edmonton Hospital campus — the largest health job in Alberta — means even peripheral work near that project builds exactly the credential KOR lacks.

3 · THE QUESTION ISLAM ASKED

Commissioning is not peer review

	PEER REVIEW	COMMISSIONING
When	During design	End of construction → occupancy
What	An independent engineer reviews <i>another firm's</i> structural design and reports on its adequacy	Verifying the <i>built</i> facility actually performs to design intent before it opens
Governed by	EGBC structural peer-review guideline; owner-driven on post-disaster and health projects	CSA Z8001 — Commissioning of Health Care Facilities; mandated by health authorities
Who leads	A senior structural engineer	Mechanical / electrical / controls specialists
Deliverable	Review report — you never stamp the design	Verification reports, functional tests, deficiency closure
Fee shape	Design-phase consulting fee	Field-heavy, smaller, recurring

The structural slice of healthcare commissioning — the lane KRA pointed at:

- **Seismic restraint and anchorage verification** of equipment and non-structural components (CSA S832 territory) — every generator, rack, ceiling, and imaging machine needs restraint review and field verification.
- **Equipment-replacement structural assessments** — MRI/CT swaps in *operating* hospitals: small fees, tight logistics, infection-control protocols, night work — recurring forever.
- **Floor vibration verification** for imaging suites and operating rooms; equipment pads, supports, openings.

Fussy, low-glamour, continuous — which is precisely why established firms avoid it, and why the seat is open.

4 · THE STRATEGIC FRAME

KRA's two comments solve each other

"We need your healthcare portfolio" and "you have a spot in commissioning" aren't two topics — they're one. KOR's healthcare portfolio is thin, and that's the entry requirement for every prime-consultant health submission KRA makes. The commissioning niche is the **fastest legitimate builder of healthcare project experience**: each small commission is a real health-facility project line, handed over by a willing architect. The graduation path runs commissioning work → named healthcare experience → the **structural design seat** on KRA's projects, in both provinces.

Take the unloved work. It converts directly into the currency KRA itself says the submissions require.

5 · ACTIONS

Five moves, named owners

ISLAM · NOW

Ask KRA the clarifying question: *"When you say commissioning — the structural scope inside your CSA Z8001 commissioning programs (seismic restraint verification, equipment anchorage, vibration checks), or structural field review generally?"* The answer decides which portfolio pieces lead and how the work is priced.

IAN + ISLAM

Assemble the healthcare portfolio this month. Lead with what KOR verifiably has — seismic/restraint work, equipment and vibration-sensitive projects, health-adjacent institutional work — in KRA's format: past relevant experience, per consultant.

IAN · NOW

Push the Fraser Health prequalification forward. KRA's warning (staged, slow) matches KOR's own prequal register — mid-process is fine; stalled is not.

ISLAM

Take the Vancouver BD manager introduction when offered and route it to the BC team — non-health BC collaboration is a separate, additive thread.

BOTH

If the answer to #1 is the Z8001 scope: propose a first small commission on a live KRA project as the pilot. The wedge matters more than its fee.

6 · HONESTY NOTES

The "commissioning spot" is KRA's characterization from one lunch — its exact scope is unconfirmed until action #1 is answered. Don't build a service line on it before that answer.

KOR's healthcare thinness is real; the portfolio should not oversell. The credible story is seismic + verification competence entering healthcare through the door healthcare firms leave open.

Everything KRA-specific beyond the lunch (project roster, people, footprint) was verified against KRA's own published pages on 2026-07-03.

Filed: lunch debrief ingested against KRA's record in KOR's BD database (2026-07-03); relationship thread bookmarked.

Sources: krarchitecture.ca (firm · people · projects) · CSA Z8000/Z8001 · CSA S832 · EGBC structural peer-review guideline · KOR BD database (KRA roster on BC/AB health capital).

Companion document: KOR-KRA-Healthcare-Channel-2026-07-03.docx (branded, in docs/).